

Hallucinations - Female (Child)

Algorithm 1106 | Child (12-17 years) | Gender at birth: Female

Definition

- UAT-only assessment of hallucinations in people aged 12 years and older, separating medical or neurological emergencies from suspected psychosis and requiring age-appropriate mental health review.
- Child patient.
- Female.

Initial assessment

1. [DURATION] When did this start, and was the onset sudden or gradual?
2. [YES_NO] Is the person confused, difficult to wake, having a seizure, severely agitated, unable to care for basic needs, or at immediate risk of harming themselves or another person?
3. [OPEN_TEXT] Is there fever, severe headache, stiff neck, recent head injury, new neurological symptoms, pregnancy or recent birth, sleep deprivation, medicine change, alcohol or drug use or withdrawal, or possible poisoning?
4. [OPEN_TEXT] What is the person seeing, hearing, feeling, smelling, or tasting, and are voices giving commands to harm themselves or someone else?
5. [YES_NO] For an adolescent, can they speak privately and safely, and is a trusted, non-threatening caregiver or responsible adult available?
6. [CAREGIVER_OBSERVATION] What change has a safe parent or caregiver observed in behaviour, alertness, eating or drinking, sleep, school or play, and normal activity?

Triage and redirect questions

1. Redirect → Poisoning

Did the hallucinations begin after a suspected overdose, poisoning, recreational drug, or unknown substance exposure?

A suspected toxic exposure requires the poisoning pathway and urgent toxicology assessment.

1. Disposition level 100

Is there immediate risk of harm; command hallucinations to harm; severe agitation or inability to maintain basic safety; sudden confusion or reduced consciousness; seizure; fever with severe headache or stiff neck; recent head injury; new neurological symptoms; suspected poisoning, overdose, or severe withdrawal; or rapidly worsening symptoms after childbirth?

Hallucinations can arise from psychosis or an acute medical, neurological, toxicological, or perinatal emergency. Immediate danger or inability to remain safe requires emergency care; sudden confusion itself requires immediate assessment. This is a direct Qatar 999 disposition and must not redirect to a missing or demographically incompatible Poisoning protocol.

1. Disposition level 78

With all emergency and acute medical features excluded, are new or worsening hallucinations present now, or is this a first suspected psychotic episode?

NHS.UK and NIMH guidance support prompt assessment of possible psychosis because hallucinations have multiple causes and early treatment is important. Children and adolescents require an age-appropriate pediatric or child mental health assessment, not automatic reuse of an adult pathway.

Advice

Qatar emergency hallucination response

- Call Qatar emergency services on 999. Stay with the person if this is safe, reduce stimulation, speak calmly in short sentences, and do not argue about or validate the hallucination as fact. Remove access to means of harm only when safe and do not allow the person to drive. If poisoning or overdose is possible, tell 999 what may have been taken; do not use an unresolved protocol redirect. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: risk of harm; command hallucinations; sudden confusion; reduced consciousness; seizure; fever or neurological symptoms; possible poisoning; person becomes unreachable.

Urgent age-appropriate mental health assessment

- Arrange prompt assessment by an appropriately trained clinician. During the HMC National Mental Health Helpline's published hours (08:00-18:00 Saturday-Thursday), call 16000 and choose option 4. An adolescent needs a private, age-appropriate assessment with safe caregiver involvement. The organization must approve the pediatric and non-emergency out-of-hours routes before UAT. Call 999 if safety cannot be maintained or any medical or neurological red flag appears.
- Call Back If: symptoms worsen rapidly; confusion or reduced responsiveness develops; command hallucinations or risk of harm develops; safe supervision fails.

First aid

- Call Qatar emergency services on 999. Stay with the person if this is safe, reduce stimulation, speak calmly in short sentences, and do not argue about or validate the hallucination as fact. Remove access to means of harm only when safe and do not allow the person to drive. If poisoning or overdose is possible, tell 999 what may have been taken; do not use an unresolved protocol redirect. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Hamad Medical Corporation, "National Mental Health Helpline marks six years of supporting Qatar's population", <https://hamad.qa/EN/news/2026/April/Pages/National-Mental-Health-Helpline-marks-six-years-of-supporting-Qatar%E2%80%99s-population.aspx> (accessed 2026-07-25)

- Qatar Ministry of Public Health, "Healthcare Services in Qatar", <https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (accessed 2026-07-25)
- National Institute of Mental Health, "Understanding Psychosis", <https://www.nimh.nih.gov/health/publications/understanding-psychosis> (accessed 2026-07-25)
- NHS.UK, "Urgent support", <https://www.nhs.uk/every-mind-matters/urgent-support/> (accessed 2026-07-25)
- NHS.UK, "Sudden confusion (delirium)", <https://www.nhs.uk/symptoms/confusion/> (page last reviewed 28 May 2024)

Provenance

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